Private Welfare in the Workplace

employee harship funds and the 'substitution hypothesis'

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06 August, 2026

Abstract

There is a long-standing claim that government social spending crowds out private savings and charity. There is also a newer set of arguments claiming that access to private resources (home equity, credit, or private charity) dampen voters' support for social insurance and redistributive spending. These claims have not been jointly investigated nor has the literature looked at workplace programs and benefits. We investigate both sides of the "substitution hypothesis" in the context of a novel set of workplace private welfare programs: Employee Hardship Funds (EHFs). EHFs enable employers to distribute emergency cash to their workers using donations pooled from employees themselves. Hundreds of major US firms now operate these programs. We use three original surveys, including survey experiments with workers at two specific employers with EHFs. We find that state-level generosity in the social safety net is unrelated to worker support for or willingness to donate to EHFs. Experimental manipulation of EHF salience among workers at two large retailers indicates that EHFs do not displace support for public unemployment insurance or government emergency aid, regardless of the generosity of the EHF program. EHFs and government social insurance programs appear unconnected in the minds of workers. Our findings show a lack of support for either side of the substitution hypothesis in the case of workplace private welfare.

For centuries, pundits and scholars have debated whether government spending crowds out private savings and charity (Townsend 1786; Friedman [1962] 2002; Andreoni 1990; Bredtmann 2023; Lehmann-Hasemeyer and Streb 2018). More recently, scholars of comparative political economy have studied how "private welfare" and "self-insurance" undermine support for social insurance and redistribution (B. Ansell 2014; Wiedemann 2022; Ahlquist and Ansell 2017; Prasad 2012; Busemeyer and Iversen 2020). Almost entirely absent from these discussions are large firms, which have tremendous leeway to introduce, revise, and eliminate everything from employee health and life insurance coverage to retirement programs to transportation and childcare subsidies to charitable

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initiatives. Does employer-based "private welfare" feed back into the political system, reducing support for government spending on these same activities? Does government welfare effort affect worker interest in firm-driven private welfare and charitable programs?

In this paper, we focus on one novel type of workplace program that combines *both* private welfare and charitable activities: Employee Hardship Funds (EHFs). These now-widespread programs—operating largely in the US—are set up by corporations as tax-exempt charities. Employers solicit donations from their own employees to fund cash grants to their workers during times of unexpected personal hardship. As essentially scaled up, tax-advantaged versions of "passing the hat," EHFs echo the long tradition of worker mutual aid. EHFs are discrete programs that vary across firms, making them tractable. We use them to study both directions of the "substitution hypothesis." As a form of corporate charitable activity, we can examine whether worker interest in and donation to EHF programs varies as a function of government welfare effort. As a form of private welfare, we can examine whether EHFs affect workers' support for government programs to aid the unemployed and those in need.

We fielded three pre-registered surveys. In an original survey of US retail workers, we ask whether variation across US States in welfare generosity predicts worker interest in and willingness to donate to EHFs. To study the effects of EHFs on workers, we fielded two original, independent surveys of workers at two large US retailers with long-standing EHF programs of widely differing generosity. We manipulate worker awareness of their employer's EHF by randomly assigning some workers to watch employer-produced or adapted testimonial videos from EHF grant recipients. We then study whether EHFs, as a form of private welfare, affect support for government programs aiding the unemployed and those in need.[1] In the end, we find that our EHF treatments do *not* reduce support for government social insurance among workers at either firm, regardless of program generosity. Even though our EHF treatment increases worker willingness to donate to the EHF, this effect is not moderated by state-level welfare generosity or individual-level considerations such as religiosity and partisanship. Using hierarchical models, we also find no detectable relationship between state-level welfare regimes and broader worker interest in EHF programs. Taken together, these findings suggest that EHFs, as a workplace mutual aid arrangement, and government social insurance programs are unconnected in the minds of workers. In short, we find no evidence consistent with either version of the substitution hypothesis.

Our findings make five contributions. First we highlight how workplace benefits have been excluded from broader debates about private welfare and the crowd out between government spending and private initiatives. Second, we show how researchers can exploit variation in workplace benefits to speak to broader social science debates. Third, we highlight EHFs as curious, understudied workplace programs. Fourth, in the case of EHFs, we fail to uncover evidence that this workplace program connects meaningfully with political attitudes. This lack of connection may be because

[1] In a companion piece (Ahlquist 2025), we study EHFs and their relationship to job attachment. That paper draws on these same data to study different outcomes. We avoid unnecessary duplication between papers but survey descriptions, case selection, and description of the experiments are nearly identical across the two papers.

EHFs are too new or too small to matter, but future research will be needed. Finally, there are policy stakes. EHFs connect to several areas of concern in the contemporary American political economy, including notable inequities across workers as well as increased employer leverage over workers in an era of monopsonistic local labor markets, weak unions, and strained safety nets. For example, EHFs, as with all workplace benefits, are conditional on employment with a specific firm, potentially exacerbating inequality across workers at different firms or in different sectors. EHFs may also increase employer power over workers when they are most vulnerable, similar to the "welfare capitalism" of the past (Brandes 1976; Jacoby 1998). EHFs, as with many job-related benefits, lack formal mechanisms for worker input and accountability, beyond worker quits. On the positive side, workers appear to value emergency aid and corporations may be more efficient at delivering immediate cash assistance than governments, at least to the families of the employed.

In the next section we go into both forms of the substitution hypothesis in more detail, highlighting the absence of any real consideration of workplace programs and benefits. Section 2 provides the necessary background on EHFs and summarizes the main hypotheses we examine here. Section 3 describes the research design and results for Study 1 while Section 4 describes the case selection, survey experiments, and results from Studies 2 and 3. Section 5 concludes.

1 The "substitution hypothesis," two ways

This paper builds on two largely disconnected literatures concerned with whether private resources and public welfare or social insurance substitute for one another. The first literature stems from arguments emerging as far back as the 18th Century (at least), claiming that government spending on poverty relief, welfare, and social insurance crowds out private charitable giving and private savings. The second and more recent literature works in the opposite direction. It focuses on the links between improved access to private financial resources—credit, appreciating assets, insurance—and reduced voter support for public welfare and social insurance programs. Both of these versions of the substitution hypothesis bear some resemblance to the policy feedback literature, long a staple of American politics, in which "policies make politics" (Schattschneider 1935; Campbell 2012). Nevertheless, the substitution hypothesis literatures we draw on are less concerned with how government policies create organized constituencies to reinforce or undermine that policy. Importantly, none of these various strands of thought has looked squarely at whether workplace benefits and charitable programs might relate to voter attitudes toward government welfare efforts.

1.1 Government spending crowds out private savings and charity

Almost as soon as governments began systematically spending tax money in support of the poor, aged, and infirm, arguments emerged that such spending is actually counterproductive.[2] The

[2]Defoe ([1704] 1893) is the first we could uncover in the British tradition.

reasons given often had much to do with purported moral failings among the laboring classes, but political economy arguments eventually emerged. Townsend (1786) may have been the first to articulate the claim that tax-funded social spending will depress *both* private savings and private charity.[3]

As the welfare state expanded dramatically in the post-War period, these arguments revived (Friedman [1962] 2002), along with empirical scrutiny. Feldstein (1974) argued that Social Security depressed household private savings by 30-50%, spawning an ongoing debate about the scale of substitution and its consequences (Alessie, Angelini, and van Santen 2013; Lehmann-Hasemeyer and Streb 2018). Several theoretical models, based on the assumption that charities provide public goods, imply that government spending will crowd out private charity to some degree (Andreoni 1990; Warr 1982; Bergstrom, Blume, and Varian 1986). Subsequent empirical work focused on determining whether government spending crowds out private giving dollar-for-dollar (as in models of "pure altruism") or at some lesser rate ("impure altruism"). A variety of empirical contexts and research designs produced widely varying estimates, including some evidence of "crowding in" (De Wit and Bekkers 2016; Bredtmann 2023; Andreoni and Payne 2013).[4] The weight of evidence suggests that government spending *can* crowd out some proportion of charity and savings, but the scale is unclear and the details matter.

Most relevant to this study, Hungerman (2005) and Gruber and Hungerman (2007) exploit variation in social support across US States (as we do below), estimating that government social welfare spending reduces charitable relief from churches by about 30%. Paarlberg et al. (2019) lack an identification strategy, but find that counties with lower tax burdens and higher proportions of Republican voters also donate more to charity. In some of the most well-identified studies, charitable organizations intervene between donations and service provision, complicating interpretation. For example, fundraising effort responds strategically to government support (Andreoni and Payne

[3]Townsend says "Let the same [generous, charitable] man be straitened in his circumstances, let him be burthened with taxes…and he will have little inclination to seek for objects of distress, or to visit the sequestered cottage of the silent sufferer…If, agreeable to the general practice of the labouring poor, a man…has made no provision for his future wants; if all, to whom he might naturally look for aid, are in the same circumstances with himself; and if the charity of those among his neighbours, who are distinguished for benevolence, nay of all who have the common feelings of humanity, is exhausted; if they who are most willing are least able to relieve him; we must expect to see distress and poverty even among those who are worthy of compassion." Malthus (1798) claims that "The poor laws of England may therefore be said to diminish both the power and the will to save among the common people" (Ch. 5). de Tocqueville ([1835] 1997) created a distinction between "private" and "legal" charity, arguing that "any permanent, regular, administrative system whose aim will be to provide for the needs of the poor, will breed more miseries than it can cure, will deprave the population that it wants to help and comfort, will in time reduce the rich to being no more than the tenant-farmers of the poor, will dry up the sources of savings, will stop the accumulation of capital, will retard the development of trade, will benumb human industry and activity, and will culminate by bringing about a violent revolution in the State, when the number of those who receive alms will have become as large as those who give it, and the indigent, no longer being able to take from the impoverished rich the means of providing for his needs, will find it easier to plunder them of all their property at one stroke than to ask for their help."

[4]Crowding-in is most frequently observed in studies of government grants to specific charities, in which government support may signal need or charity quality, a line of reasoning irrelevant to this paper. A more relevant possibility is that both government spending and private charity are signals (or byproducts) of broader cultural or institutional dispositions toward collective support (De Wit et al. 2018). Lab experimental studies tend to provide the largest estimates of crowding out.

2011). Panjwani and Xiong (2023) show that health-related crowdfunding campaigns (i.e., demand for charity) declined after expansions in medical insurance coverage due to the Affordable Care Act. Lee and Lehdonvirta (2022) show that areas with poor private insurance coverage and fewer social associations see greater crowdfunding campaign initiation rates, but campaigns in these same areas are *less* likely to be successful (i.e., charitable capacity or norms might be weaker). Bernard and Gazel (2025) show that crowdfunding projects with low excludability (i.e., you do not need to donate to enjoy benefits, should the project go forward) exhibit the most crowding out. EHFs are non-excludable in this regard.

In attempting to estimate the crowd out rate, existing empirical work frequently uses actual government social spending, as opposed to indicators of the spending "regime" (Esping-Andersen 1990). This is a problematic conflation for at least three reasons. First, spending levels at any point in time are endogenous to conditions that may also affect savings and charitable giving.[5] Second, citizens' beliefs about government spending in specific areas are only weakly connected to actual spending levels (which fluctuates more than opinion) (De Wit, Bekkers, and Broese van Groenou 2017). Voter knowledge about the level of government support directed to particular intermediary charities is even more meager (Horne, Johnson, and Van Slyke 2005). Voter beliefs about government spending levels, however, are connected to their pre-existing partisan identities (Bullock et al. 2015; Mettler 2011). Klein Teeselink and Melios (2025) show that co-partisans of the US President reduce their charitable donations independently of any changes to the level and composition of actual spending, likely because they are more confident in the efficacy of government when their preferred party is in power. Third, expectations about the adequacy of *future* spending should matter when forming opinions or making donations. As such, the adequacy and credibility of social spending *regimes* would be more relevant than actual spending levels at any moment in time, as Esping-Andersen (1990) noted decades ago.[6] We take this on in our measurement choices below.

Firms are significant sources of charitable funding across a range of causes. This funding can take the form of direct corporate donations to specific charities or even individuals. But firms also actively organize their employees to donate their private resources, sometimes using matching or other incentives (Meier 2007). Firms appear to give for different reasons than individuals (Brammer and Millington 2005; Bertrand et al. 2021), but workplace philanthropic fundraising is significant (Rimes, Nesbit, and Christensen 2019; Shaker and Christensen 2019). Political scientists have studied employer-organized political donations (Hertel-Fernandez 2018; Stuckatz 2022), which do appear to partially crowd out other charitable contributions (Yildirim et al. 2024). But, as yet, there is no work examining any link between the government safety net and workplace giving, especially when the beneficiaries of workplace charity are other workers. We provide the first such test looking at EHF programs.

[5] There are several clever empirical strategies for addressing this, but they produce different estimates of crowd out from very different contexts (Boberg-Fazlić and Sharp 2017; Hungerman 2005; Gruber and Hungerman 2007; Andreoni, Payne, and Smith 2014)

[6] "Expenditures are epiphenomenal to the theoretical substance of welfare states." p.19

1.2 Private resources crowd out support for a public safety net

A newer, smaller, and largely parallel literature has emerged, documenting how shocks to private resources affect voter support for public social programs. Most obviously, governments sometimes step in when private resources are diminished. Margalit (2013) uses panel data around the Great Recession and finds that people who lose their job become more supportive of government support for the unemployed, but when the need dissipates, so does their support for government unemployment insurance, especially among Republican voters. As an historical matter, government administered, tax-funded unemployment insurance grew out of the repeated collapse (and ad hoc bailouts) of privately-organized welfare and mutual aid pools (Administration 2023; Western 1997; Harris 2018; Glenn 2001).

More attention has been paid to how improvements in private resources can crowd out support for government social insurance and welfare programs. The logic is simple: voters who perceive an improved financial position are more confident in their ability to "self-insure" against negative income shocks while also becoming less willing to pay taxes into the public system from which they perceive a reduced benefit. Real estate (especially housing) is a key, politically relevant asset and source of savings (B. W. Ansell 2019). B. Ansell (2014) finds that American and British voters with appreciating real estate assets are less supportive of social insurance. Wiedemann (2022) and Ahlquist and Ansell (2017) focus on easy access to credit (related to home ownership and real estate prices) as a key private resource that can reduce pressure for redistributive policy and social spending. Where real estate is more financialized (i.e., more tied to credit markets), home owners are less supportive of redistributive social policies (André and Dewilde 2016).

Beyond housing and credit, economic risk and access to insurance has been a major area of interest. Those able to "self-insure" through higher wealth and incomes are less worried about negative economic shocks and less supportive of government spending to help the unemployed, infirm, and aged (Rehm, Hacker, and Schlesinger 2012; Hacker, Rehm, and Schlesinger 2013). Busemeyer and Iversen (2020) provide a theoretical account and empirical data illustrating how the presence of private insurance options reduces high-income voters' support for unemployment and pension spending. Increasingly fine-grained data combined with growing income inequality increase the scope for private insurance options to emerge, weakening support for "solidaristic" government policies (Iversen and Rehm 2022).[7]

Private charity is also relevant here. Werfel (2018) uses survey experiments to manipulate beliefs about the degree of private charitable funding directed at specific needs. In the area of social services, greater perceived charitable activity significantly reduced support among conservatives and moderates for government spending in that same area.

Again, however, there is a dearth of research that explores how variation in employer-provided

[7] Hadziabdic and Kohl (2022), using German panel data, finds no robust relationship between taking out life insurance and broad attitudes around social spending and government activity.

amenities and insurance as well as corporate charitable efforts affect voter attitudes and behavior. This is remarkable, as there is considerable variation across firms and over time and space in workplace amenities as well as corporate charitable initiatives. Insofar as EHFs are a form of private emergency aid, increased program awareness could blunt support for social insurance or government aid programs in times of emergency (e.g., COVID-19). This paper seeks to determine whether we can observe this effect among US retail workers.

2 Employee Hardship Funds

2.1 Background

In the contemporary United States, unions are vanishing and the welfare and social insurance systems are creaking. Temporary and other "non-standard" employment relationships are increasingly widespread (Abraham et al. 2021; Weil 2014). Yet such work arrangements fail to provide access to traditional social insurance and labor market protections (Thelen 2019). Americans in the lower half of the education and earnings distributions have seen years of stagnating wages while also bearing more economic risk (Hacker 2019).

Against this backdrop, The Home Depot has maintained the Homer Fund since 1999 while Walmart started the Associates in Critical Need Trust (ACNT) in 2001.[8] Both programs are set up as nonprofit charities (501c(3)) offering cash grants for employees facing emergency expenses. The Homer Fund makes grants of up to $10,000 while the Walmart fund makes grants of up to $1,500. Beyond simple generosity, the programs differ in multiple ways, including application procedures and the frequency with which a worker can receive a grant. Both funds solicit (tax deductible) donations from both corporate and front-line employees. Grants to workers are tax-exempt.

Both ACNT and the Homer Fund are examples of Employee Hardship Funds (EHFs). By blending aspects of charities and private "welfare capitalism" EHFs belie the extraordinary leeway that employers have to introduce job amenities and programs that may both respond to and affect government policy. Nevertheless, these programs have gone largely unstudied. Outside of a small practitioner literature, there is very little accessible information about EHFs (Association of Disaster Relief Funds and Employee Hardship Funds 2013; Group 2019; Institute 2019; Rodriguez 2020b, 2020a).[9] There is no central database of EHFs, but we have positively documented over 320 companies across multiple industries and sectors operating EHFs as of January 2025. Nine of the ten top US retailers by revenue have maintained an EHF. There is sufficient corporate interest in these programs that a cottage industry in EHF management has developed. Many employers now outsource the administration of their EHFs to third-party nonprofit agencies, the largest of

[8] ACNT has since renamed as the ACNT Together Fund.

[9] Amorim and Schneider (2022); Reich and Bearman (2018) are the only two peer-reviewed pieces mentioning EHFs of which we are aware.

which appears to be the Employee Assistance Foundation (EAF), which claims to have over 400 corporate clients. EAF distributed over $86 million in grants in 2020 according to IRS filings.

EHFs appear to be expanding. Levi Strauss & Co. reported a three-fold increase in applications to their EHF in 2020 (Rodriguez 2020a), with another spike in applications once coronavirus emergency aid programs expired in the US. The Home Depot suspended fundraising from employees and expedited grant processing during the pandemic emergency. EAF reported a more than tenfold increase in grant applications relative to historical averages in the first two quarters of 2020 as well as 140 new client EHFs (Foundation 2022). In May 2020 Amazon.com launched its EHF—using $25 million in corporate money—in the face of extensive criticism of its workplace safety practices during the coronavirus outbreak as well as ongoing attempts to unionize its warehouse and fulfillment centers. Amazon shuttered its EHF in August 2022.

Figure 1 zooms in on the Home Depot's Homer Fund and Walmart's ACNT using data from IRS 990 forms (Lecy 2023; Suozzo et al. 2025). In the left panel we report net assets for each firm's nonprofit foundations.[10] Both corporate charities show strong growth from 2010, but Homer Fund is operating at more than double the ACNT's size. The right panel looks at grants to domestic individuals, my preferred indicator of EHF activity.[11] Homer Fund is making $15-25 million in annual grants between 2008 and 2022 whereas ACNT typically disburses less than $10 million, notwithstanding Walmart's much larger corporate presence. Homer Fund reports making a total of over $267 million in grants to more than 183,000 individuals over its existence. ACNT claims to have disbursed over $142 million over its existence to over 145,000 workers.

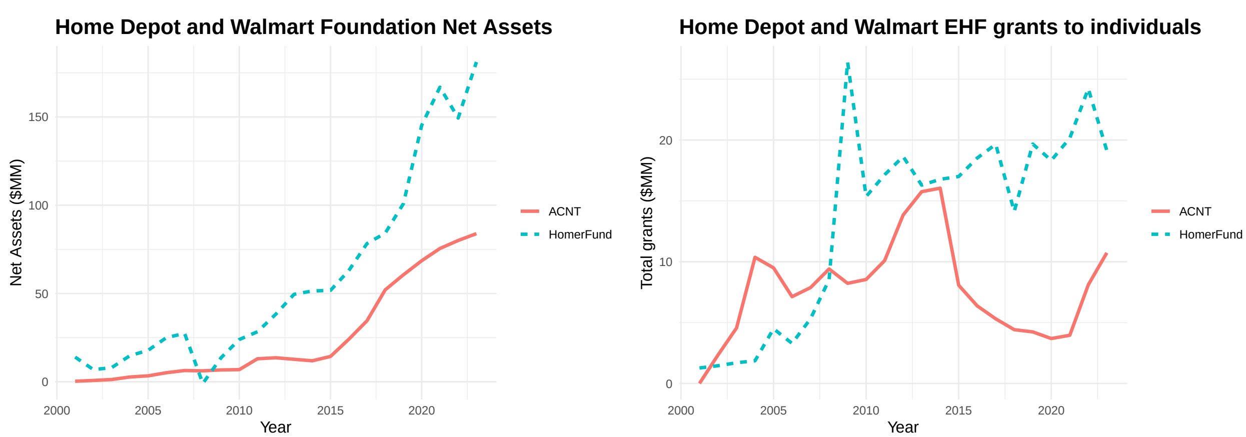


Figure 1: EHF assets and grant activity at the Home Depot and Walmart.

The examples above illustrate how EHFs vary across firms in virtually all programmatic aspects including longevity, eligibility requirements, application procedures, grant generosity and frequency,

[10] For tax purposes, both Homer Fund and ACNT EHFs are part of the larger Home Depot and Walmart corporate foundations, respectively.

[11] The larger foundations typically focus on disbursing grants to other nonprofit and community organizations, as opposed to individuals. The Homer Fund website reports disbursing $17 million in 2023 EHF grants whereas the Home Depot foundation 2023 IRS form 990 reports $19.1 million in grants to individuals.

funding structure, and even whether they are managed in-house or out-sourced to third parties. Although all EHFs have a mutual aid component, communication around the programs varies in tone and intensity. The Home Depot aggressively promote its EHF through dedicated social media and YouTube channels as well as regular donation drives. Walmart appears to put substantially less effort into promoting their program.

2.2 Hypotheses

EHFs combine charitable and solidaristic motivations with job amenities and the appearance of social insurance. Workers are both asked to donate to these funds while hearing that their employer intends to help in times of unexpected hardship. Moreover, the structure and generosity of these programs varies significantly across employers. Connecting EHFs to both forms of the substitution hypothesis is straight forward. If the substitution hypothesis holds, we expect workers in places with stronger government safety nets to be less interested in and less willing to donate to EHFs. Existing work suggests that political conservatives may be more attentive to these ostensible trade-offs. In the US, there is considerable variation across states in the generosity of unemployment and welfare benefits as well as in the administrative burdens workers face in accessing benefits when needed. We exploit this cross-state variation below.

In the other direction, workers exposed to information about their employer's EHFs will be less supportive of government programs to support people in times of hardship, as their employer is now intervening. The more generous the EHF, the less a worker may perceive a need for public programs.

We collect these hypotheses in Table 1 for ease of reference.

Table 1: Summary of Empirical Hypotheses

Hypothesis	Description	Study
H1	Workers in states with more generous social spending regimes will be less supportive of EHFs and less willing to donate.	1, 3
H1a	The correlation with state spending will be stronger among conservatives	1
H2	Exposure to EHF messaging reduces support for government spending on social insurance and welfare.	2, 3
H2a	Effects should be bigger among those unaware of the EHF before treatment.	2, 3
H2b	Effects should be bigger where the EHF is more generous.	2, 3

3 Study 1

In Study 1 we explore hypotheses 1 and 1a. We fielded an original survey of US retail workers with questions probing their knowledge and experience with EHFs, including whether their current employer has one and their level of support for these programs. We focus on a single industry to hold fixed cross-industry variation. Retail is attractive as it is employs over 15 million Americans, with that number growing in the last decade. Retail work is also known for lower wages and levels of human capital development along with unpredictable scheduling, suggesting that retail workers might value EHF-style benefits or insurance.

We fielded the survey using the Cloud Research incentivized panel, targeting US respondents 18+ employed in retail. The survey was in the field from February 12 - 22, 2025 and produced 1008 valid responses.[12] Using the 2024 American Community Survey-reported demographics for US adults working in retail, we constructed post-stratification weights based on gender, race, and education. Descriptive statistics about the sample appear in Appendix B.

3.1 Research design

In the US, virtually all social safety net and labor market benefits for working age citizens are managed at the state level.[13] Consequently, there is significant variation across states in both the generosity of the social safety net and the ease of accessing the benefits that are available (Herd and Moynihan 2019; Fox, Feng, and Reynolds 2023). For example, in normal economic times, most states offer up to 26 weeks of unemployment insurance benefits, with some states offering as little as 12 while Montana provides 28. The main cash welfare program, Temporary Assistance to Needy Families (TANF), provides a maximum monthly benefit of $915 in New Hampshire but only $162 in Arkansas for a single parent of one child of in 2023 (Falk and Landers 2024).

In Study 1, we examine whether state-level safety net generosity is a meaningful predictor of EHF support and donation intentions. We rely on cross-state variation in the robustness of their social safety nets to look at two specific outcomes: the degree of support for an EHF at the respondent's current retail employer and the stated willingness to donate to the EHF. Both outcomes are ordered categorical variables.[14] Question wording varied slightly based on whether a respondent reported an employer with an EHF.[15] We therefore condition on whether the respondent claims to have an EHF.

[12] Of those targeted, only respondents with US IP addresses, who passed a screening question confirming employment in the retail industry, and who successfully passed two different attention check questions were allowed to complete the survey. Respondents with duplicate IP addresses reporting identical demographic values were also excluded.

[13] Social Security, Medicare, and veterans' benefits are the primary programs that are run directly by the federal government.

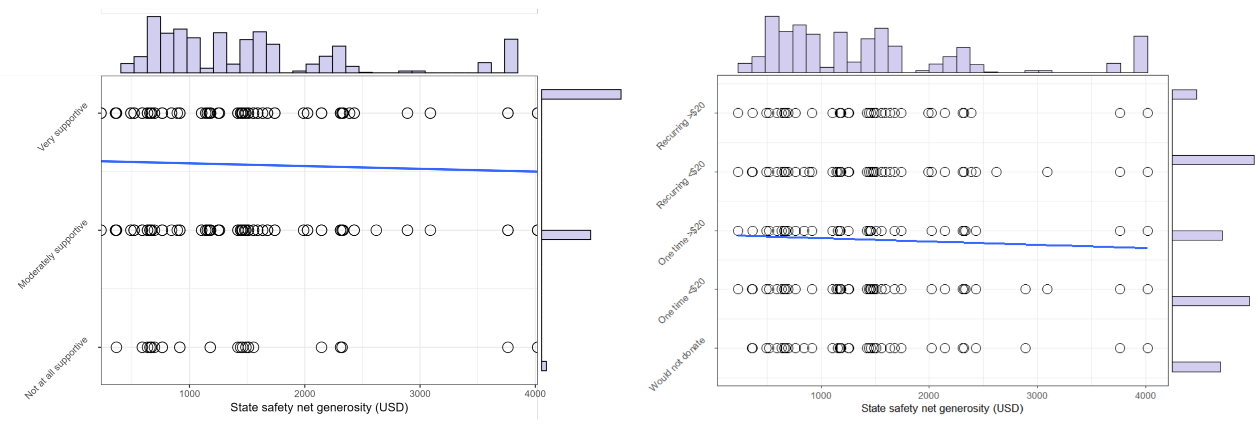
[14] Outcome categories for the donation question were {nothing at all, one time donation less than $20, one time donation more than $20, regular donation of less than $20, regular donation of more than $20}.

[15] We were not in a position to verify whether respondent reports were accurate, so this represents perceptions only.

In measuring variation in the state-level safety net, we rely on the standardized benefit levels produced by Schmidt, Shore-Sheppard, and Watson (2024). In our main specifications, we use the "state directed" portion of these benefits, to better isolate differences across states.[16]

3.2 Results

Figure 2 displays the distributions of both EHF outcomes against safety net generosity, measured in US dollars. Inspection of the graphs suggests that there is no relationship between safety net generosity and either support for or willingness to donate to an EHF.



(a) Support

(b) Willingness to Donate

Figure 2: EHF support and willingness to donate to a potential new EHF, by state level welfare benefits in USD. Marginal distributions of the variables are included as blue histograms.

We explore these relationships further using both linear regression and hierarchical linear models allowing for random intercepts as well as cross-state variation in the effect of political conservatism, both as a function of state-level safety net generosity. In addition to state safety net variables, we also include a variety of other covariates connected to charitable giving propensity and the substitution hypothesis. These include whether the respondent has used welfare in the past, religiosity, and whether the respondent identifies as politically conservative (Brooks 2006; Bekkers and Wiepking 2011b; Wiepking and Bekkers 2012; Andreoni and Payne 2013).[17]. Because appreciating real estate assets may depress support for the welfare state, we produce a measure of home value increases. Specifically, we multiply an indicator for home ownership with the 5-year change in the state-level home price index produced by the US Federal Housing Finance Agency. We include both terms of

[16]Our findings do not hinge on this decision. Using the "cash and food" benefits estimates in Schmidt, Shore-Sheppard, and Watson (2024) or the measure of TANF benefit generosity from Fox, Feng, and Reynolds (2023) produce similar results. Measure of UI generosity, including maximum weeks of benefits and average weekly benefit amounts also show no relationship with EHF support or donation.

[17]Religiosity is a binary variable measuring if a respondent declared affiliation with any religion. Conservatism is also a binary variable, measuring whether the respondent placed themselves as "Conservative" or "Leaning Conservative" in a question about ideological identification.

this product in Model E. We also condition on race, gender, age, education, and household income but do not report those for space considerations.

In Tables 2 and 3 we present five models, of increasing complexity, that regress EHF support and willingness to donate to an EHF on state level generosity. The first two models are cluster robust standard error OLS models, with standard error clustering at the state level. Model A includes a suite of pre-registered covariates.[18] Model B additionally includes household income, whether the respondent identifies as politically conservative, religiosity, and home value appreciation calculated using the Federal Housing Finance Agency's House Price Index.

There is little evidence of correlation of state safety net generosity with support or willingness to donate to an EHF. Between the four estimated models, only Model B shows a statistically significant and negative effect ($p < 0.05$ for the donation outcome, $p < 0.1$ for the support outcome). This, in addition to the improving performance of Model B in terms of AIC, may indicate some omitted variable bias present in Model A that is accounted for by the inclusion of additional covariates in Model B.[19] There is, however, reason to question the importance of this finding in relation to our hypotheses. First, the magnitude of the point estimate is extremely small in comparison to the outcome variable, equal to approximately 0.05 of one standard deviation. It is also around a third of the effect of having previously received welfare. Second, this finding is not consistent with any other of the eight fitted models, and does not reach conventional levels of statistical significance for the EHF support outcome.

We then fit three additional hierarchical linear models (HLMs) which should be able to detect any state level variation either in the intercept or the slope of key variables, and compare that to the unexplained residual variance at the individual level. Large variances in the intercept term (relative to residual variance) indicate that, even after conditioning on all included variables, there is substantial unexplained variance at the state level. Large variance in the slope of the key variables would mean that the relationship between the key variable and the outcome is highly heterogeneous across states. If either of these are the case, then this could indicate some uncaptured variance at the state level that explains support for or willingness to donate to an EHF. That would be a potential threat to our conclusions, if the state-level variation is correlated with some unmeasured aspect of the state-level safety net.

All three models include the expanded set of covariates present in Model B. All models are estimated using restricted maximum likelihood methodology, which compensates for downward bias present in typical maximum likelihood estimation. Model C adds a random intercept term, which partially pools state-level intercept estimates. Model D adds a random slope estimate for the effect

[18] These are age, gender, race, college degree status, tenure at their job, whether this job is their primary job, and whether they work full-time. Ahlquist (2025) shows that job tenure and full-time status are the primary predictors of pre-exposure to the EHF.

[19] This result is not driven by the slight decrease in sample size between Model A and the remaining four models. We run Model A on the support of Model B and find no difference in the reported results. Full regression tables for the limited models are omitted for parsimony.

Table 2: Support for EHF among retail workers by state level safety net generosity models. There is no detected state level variation.

	Model A	Model B	Model C	Model D	Model E
State safety net generosity	−0.010	−0.012*	−0.013	−0.015	−0.009
	(0.007)	(0.007)	(0.010)	(0.011)	(0.010)
Has an EHF	0.023	0.023	0.022	0.023	0.023
	(0.020)	(0.021)	(0.023)	(0.023)	(0.023)
Previous welfare recipient	0.065***	0.064***	0.064***	0.065***	0.067***
	(0.019)	(0.019)	(0.020)	(0.020)	(0.020)
Conservative		−0.014	−0.014	−0.029	−0.017
		(0.021)	(0.023)	(0.040)	(0.023)
Generosity x Conservative				0.010	
				(0.023)	
Home owner					−0.056
					(0.067)
Home owner x house appreciation					0.000
					(0.001)
Var: intercept			0.019	0.018	0.271
Var: house appreciation					0.003
Var: conservative				0.004	
Var: residual			0.277	0.278	0.275
Model Type	Cluster-robust	Cluster-robust	HLM	HLM	HLM
N	1008	988	989	988	988
AIC	288	293	406	434	453

* p <0.1, ** p <0.05, *** p <0.01
Standard errors in parentheses. Additional covariates include age, gender, race, job tenure, hourly and full time status, college degree, whether the reported employment is their main job, income, religiosity.

Table 3: Willingness to donate to EHF among retail workers who do not have an EHF, by state level safety net generosity models. There is no detected state level variation.

	Model A	Model B	Model C	Model D	Model E
State safety net generosity	−0.011	−0.018**	−0.018	−0.014	−0.018
	(0.008)	(0.007)	(0.011)	(0.013)	(0.011)
Previous welfare recipient	0.060***	0.061***	0.061**	0.062**	0.061**
	(0.023)	(0.022)	(0.024)	(0.024)	(0.024)
Conservative		−0.021	−0.021	0.017	−0.023
		(0.033)	(0.029)	(0.060)	(0.029)
Generosity x Conservative				−0.029	
				(0.035)	
Home owner					0.033
					(0.081)
Home owner x house appreciation					−0.001
					(0.001)
Var: intercept			0.000	0.029	0.101
Var: house appreciation					0.001
Var: conservative				0.090	
Var: residual			0.304	0.302	0.303
Model Type	Cluster-robust	Cluster-robust	HLM	HLM	HLM
N	811	792	792	792	792
AIC	391	377	496	505	520

* p <0.1, ** p <0.05, *** p <0.01
Standard errors in parentheses. Additional covariates include age, gender, race, job tenure, hourly and full time status, college degree, whether the reported employment is their main job, income, religiosity.

of identifying as a conservative on EHF support/willingness to donate, as well as a fixed-effect interaction term between generosity and conservative self-identification. Model E is similar to Model D, but replaces conservative identification with home price appreciation.

The majority of non-residual variances estimated by the HLMs are close to zero, which given the [0, 1] range of the response and the magnitude of the residual variance estimate, signify that no additional variance is explained by the hierarchical structure. The exception to this is the estimated state-level variance of the intercept term in Model E. However, a profile-likelihood confidence interval on the variance components of Model E shows the intercept variance is not distinguishable from zero (95% CI: 0 – 0.195 for donation, 0 - 0.261 for support) and the random slope on house price appreciation is negligible, with the correlation between them unidentifiable. This is consistent with a near-singular fit, rather than genuine state-level variation. When examining model performance, as our models become more complex, AIC values increase, indicating that the overall model fit is deteriorating, which would not be the case if the models were capturing substantial state-level variance.

The results, therefore, indicate little evidence in favor of either version of the substitution hypothesis. There is little to no variation in terms of state level effects on EHF support and willingness to donate that is explained through State-level variation in benefit generosity. Given this fact, and the findings of models B and D, we can also conclude that ideological identification as conservative also does not affect the correlation between State-level generosity and EHF support.

4 Studies 2 & 3

4.1 Case selection

Studies 2 and 3 focus on workers at two "big box" US retailers with well-established EHF programs, The Home Depot and Walmart. These firms compete to some degree in both product and labor markets. By looking at two large firms in the same industry, we can hold industry considerations constant. We focus on these two firms for four reasons. First, they are both extremely large employers, making our independent survey recruitment strategy feasible (more below). Second, by concentrating on two specific firms we can conduct survey-based experiments tailored to specific EHF programs. Third, both firms have in-house EHFs of similar vintage. Fourth, the Home Depot and Walmart EHFs differ markedly in both their generosity and in the level of worker engagement, allowing us to compare findings across contexts to see whether program differences matter. We briefly outline the two programs.

The Home Depot's EHF—The Homer Fund—is managed in-house and has been in continuous operation since 1999. The Homer Fund is among the most generous EHFs, with a maximum grant amount of $10,000 per event, eligibility from the first day of employment, and the ability to apply repeatedly (conditional on a new qualifying event). The Homer Fund is sizable, reporting disbursements of $21.2 million to over 12,000 workers in the 2022 fiscal year (Depot 2023), the year of the survey. Importantly, the Home Depot aggressively promotes the Homer Fund to encourage worker engagement, especially donations. The Homer Fund has a dedicated website and YouTube channel with numerous professional-quality testimonial videos, along with active social media accounts aimed at workers. The Home Depot runs regular Homer Fund donation drives in which different stores compete with one another.

As with the Homer Fund, Walmart's ACNT Together Fund is over 20 years old and managed in-house. The EHF is available to both full- and part-time workers who have been at Walmart for at least 90 days. But the ACNT is substantially less generous than the Homer Fund, with a maximum grant of $1500 in a rolling 5-year period, regardless of how many qualifying events occur. The ACNT Together Fund has minimal presence on social media and there is no evidence of any donation drives or promotional efforts similar to those at the Home Depot. Interestingly, Walmart matches employee donations to the Walmart political action committee 2:1 with corporate donations to the Together Fund.[20]

4.2 The samples and surveys

To independently construct matched worker-employer survey samples, we recruit participants using ads on the Meta platforms (Facebook, Instagram, Threads) targeted at users 18 and older with Meta-reported status as Home Depot and Walmart employees, respectively (Schneider and Harknett 2019). Those completing the survey were offered entry into a raffle for a free iPad Mini or Visa gift card. Schneider and Harknett (2019) show that this strategy has considerable promise, producing accurate estimates of the employee population in the retail industry. But the social media recruitment strategy has the obvious drawback of potentially biased sample selection. Selection concerns are unavoidable to some degree, but Schneider and Harknett (2019) demonstrate that this strategy can still recover good estimates of known firm-level quantities. As we are interested in in-sample treatment effects and we condition on potential weighting variables, we do not use any survey weights in the analysis.

Survey recruitment for the Home Depot sample occurred between 7 September and 15 October 2021. In the end, 515 individuals completed the survey, passed an attention check, and confirmed their Home Depot employment status. The Walmart survey was in the field from October 23 to December 4, 2024 and produced 980 complete responses that passed similar screening and attention checks. Summary statistics for all variables by treatment condition appear in Appendices D, and E.

The surveys in Studies 2 and 3 were very similar in question wording and ordering, but Study 3 had several extensions to Study 2, including a behavioral measure of donation to the EHF (clicking

[20] In 2015, the Federal Election Commission rejected a challenge to this practice. In our sample, fewer than 1% of respondents reported making a contribution to the Walmart PAC.

on a link to a donation page). Study 3 also included an expanded the EHF survey experiment that included a placebo video condition.

4.3 EHF survey experiments

In both studies 2 and 3, we randomly exposed respondents to messages about their employer's EHF to test H2, 2a, and 2b. The main quantities of interest are the average treatment effects (ATE) for each of the EHF treatments relative to control for each of the outcome variables; specifics on the treatments and outcomes of interest are presented in the following sections. For each, we present graphical displays as well as regression-based estimates.

Importantly, we are randomly assigning information, not program access or benefits. Some workers arrive in the survey with prior exposure to information about their employer's EHF. These "pre-exposed" may be less responsive to the treatment messages than those unaware of the EHF prior to the survey.[21] To the extent we are interested in the effect of informing a previously uniformed worker about the EHF, the ATEs could be subject to "pre-exposure bias" (Druckman and Leeper 2012; Ferrari 2023). We therefore consider heterogeneity in treatment effects based on pre-exposure to the EHF.

We measure prior awareness of the employer's EHF by presenting them with a list of job amenities and asking them to select all of the ones they have access to. We code a positive response to "emergency cash to help in times of need" as indicating awareness of the EHF program.[22] We ask this question of all respondents before they encounter the experimental treatment. The assumption needed to sustain a causal interpretation of the treatment is that pre-exposure to information about the EHF is independent of potential outcomes, conditional on treatment and relevant pre-treatment covariates.[23]

In the analysis that follows, we report four models for each outcome. The first includes only the experimental treatment indicators, providing ATE estimates. The second adds a treatment-by-awareness interaction term. The third includes a suite of pre-registered pre-treatment covariates. The fourth, "expanded" model then adds additional covariates that were not pre-registered but that are relevant to the substitution hypothesis literatures: household income, whether the respondent identifies as politically conservative, religiosity, safety net generosity in the respondent's state of residence (Schmidt, Shore-Sheppard, and Watson 2024), and home value appreciation calculated using the Federal Housing Finance Agency's House Price Index. Following the pre-registered plan, we report linear regression models in the main text, employing robust or clustered standard errors, as appropriate. Models employing different distributional assumptions appear in the appendix.

[21] We use the terms "aware" and "pre-exposed" interchangeably.

[22] See Ahlquist (2025) for a detailed discussion of this measurement strategy and comparison with other measures of EHF awareness in the survey.

[23] Pre-exposure can also be viewed through the lens of compliance with treatment assignment; the pre-exposed are "always takers" while the un-exposed are "compliers". Since we have direct measures of pre-exposure, however, the heterogeneous effects approach is more natural.

4.3.1 Study 2

The experiment in Study 2 consisted of a control condition and two treatment arms, referred to as the *text* and *video* treatments, respectively. Respondents in the control arm saw no prompt or mention of the Home Depot EHF. Those in the text treatment saw a brief, neutral statement describing their employer's EHF.[24]

Neutral textual descriptions do not reflect how employers actually communicate with their employees. Fortunately, the Home Depot maintains a YouTube channel that includes high-quality video testimonials from purported Homer Fund grant recipients. Study 2 respondents in the video treatment were asked to watch the video displayed in Figure 3, panel (a).[25]

Our key outcomes of interest in Study 2 comes from a battery of questions asking respondents to report how much responsibility the government should have for ensuring the welfare of different groups, with responses in four categories ranging from "none at all" to "a lot". Here we examine responses for "the unemployed". This question format, taken from the General Social Survey, is also part of the ISSP "role of government" module and is widely used for gauging attitudes toward government social insurance and welfare activities (Blekesaune and Quadagno 2003; Bean and Papadakis 1998; Rehm, Hacker, and Schlesinger 2012; Deeming 2018). We will interpret responses to this question as reflecting support for government-provided unemployment insurance (UI).

4.3.1.1 Attitudes toward social insurance The left panel of Figure 4 displays the distribution of responses by EHF treatment status for Study 2. Overall, workers in the Home Depot sample are quite favorable toward government support for the unemployed, with a solid majority viewing the government as at least somewhat responsible for the welfare of the unemployed. The video treatment clearly *reduces* the proportion of responses in the "none" category in favor of the two positive categories. In other words, treatment made respondents *more* rather than less supportive of government-provided social insurance, contrary to the substitution hypothesis. The text treatment, on the other hand, does not appear to have any detectable treatment effect on average.

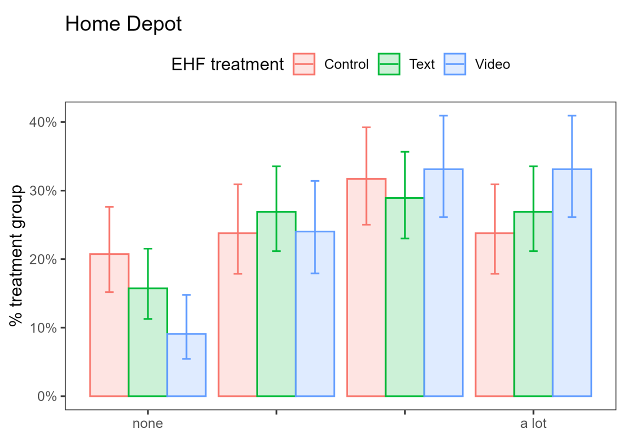
[24] The prompt was "Your employer, The Home Depot, maintains a program called the Homer Fund. The Homer Fund combines donations from workers like you with money from The Home Depot corporation. The Homer Fund uses this money to offer cash grants of up to $10,000 to Home Depot employees in times of unexpected financial hardship like a natural disaster, illness, or death in the family."

[25] The video can be found here: https://www.youtube.com/watch?v=qF-HBoySMe and here: https://corporate.homedepot.com/news/foundation-and-community/homer-fund-celebrates-20-years-giving-rays-story. The summary text appearing on the Home Depot website is: "When Ray, a retired Army veteran and proud Home Depot associate, fell severely ill, his family needed help covering expenses such as food and gas. With the help the Homer Fund and his fellow associates' generosity, he and his family were able to recover. The Homer Fund, a nonprofit exclusively for Home Depot associates, celebrates 20 years of giving in 2019. Established by our co-founders Bernie Marcus, Arthur Blank and Ken Langone, it has awarded $176 million to more than 138,000 Home Depot families facing unforeseen hardships. For Ray and his family, The Fund's help came at a critical time. Ray spent six weeks in an induced coma. The Homer Fund stepped in to help his family secure necessities so they could focus their energy on Ray's recovery. The Fund's impact on Ray's family reminded him of the comradery of his Army days. 'You know you have a brother and sister to your left or to your right. And you look out for them and they look out for you,' Ray says." This text was not presented to survey respondents, but is presented here to provide a summary of video contents. The video was 2:41 in length.

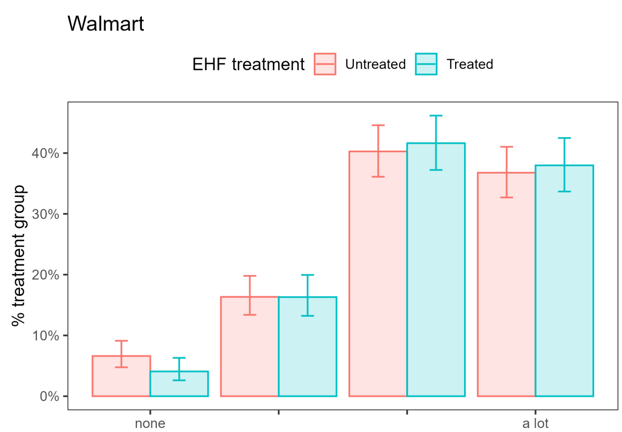
(a) Home Depot

(b) Walmart

Figure 3: Screen stills of the video treatments



(a) Study 2



(b) Study 3

Figure 4: Government responsibility for supporting the unemployed, by EHF treatment status. Vertical bars are 95% confidence intervals.

In Table 4, we display estimates from a series of linear regression models on UI support. The first model, including only experimental treatments, confirms a positive average treatment effect for the video treatment of about 0.31 standard deviations. Looking at the second model that accounts for pre-exposure, we find that respondents pre-exposed to the Home Depot EHF are -0.3 standard deviations less supportive of UI than the unaware ($p < 0.1$), but we find no treatment effects among the unaware. Contrary to the substitution hypothesis, we do find positive treatment effects among the *pre-exposed*, especially for the video treatment. Adding pre-registered covariates, including those that predict EHF pre-exposure (the third model), does not meaningfully alter interpretation. The final model in Table 4 includes a whole series of predictors connected to the substitution hypothesis, entered as linear terms.[26] Unsurprisingly, more conservative respondents are less supportive of government support for the unemployed. Income, past use of welfare benefits, religiosity, and increased home values are all unrelated to support for UI. But those living in state with more generous safety nets are also more positive about government support for the unemployed, even after conditioning on conservativeness, suggesting that unemployment insurance and the broader safety net are connected in respondents' minds.

To interpret the heterogeneous treatment effects more easily, Figure 5 displays treatment effects by respondent pre-exposure to the EHF, derived from the second model. In the case of support for the unemployed, we see that treatment effects are entirely concentrated among those *already* aware of the Home Depot's EHF. Contrary to expectations, both the text and video treatments *increase* support for UI, although only the latter is a significant effect by conventional standards. This finding deserves more exploration, but it is consistent with author interviews with EHF professionals who indicate that EHFs are meant to supplement or provide a bridge to public benefits, rather than supplant them.

Is unemployment support the appropriate policy to link with the EHF? After all, the EHF is designed to provide benefits to individuals who have a job with the respective employer, whereas UI is designed to help when a job is lost.[27] There are two pieces of evidence that UI is a reasonable policy domain to link with EHF benefits. First, although a Home Depot worker who loses her job is no longer eligible for EHF benefits from that employer, workers are eligible for EHF grants when an immediate family member loses their job. Second, the UI question was asked as part of a battery of questions asking about UI was well as two other policy areas: government support for the elderly and the provision of childcare for working parents. These two questions can constitute "placebos" since EHFs have no connection with either policy area. Parameter estimates from regressing support for these alternative policy areas on the EHF treatments and EHF awareness appear in Appendix I. There is no evidence for any relationship between the EHF treatments and support for government involvement in either elder support or childcare, regardless of whether the respondent was aware of the EHF going into the survey and adjusting for predictors of awareness

[26] There was no evidence that the the treatment effect was moderated by state-level welfare generosity or respondent conservativeness.

[27] The pre-exposed may better appreciate this distinction, which may help explain the unexpected findings.

Table 4: Government support for the unemployed (Home Depot)

	Base	Pre-exposure	Pre-registered	Expanded
Text treatment	0.032	−0.053	−0.057	−0.073
	(0.037)	(0.062)	(0.062)	(0.055)
Video treatment	0.108***	0.031	0.032	−0.009
	(0.038)	(0.060)	(0.059)	(0.059)
Pre-exposed		−0.105*	−0.102*	−0.090**
		(0.057)	(0.055)	(0.044)
Text x pre-exposed		0.141*	0.148*	0.142*
		(0.078)	(0.076)	(0.080)
Video x pre-exposed		0.127	0.116	0.117*
		(0.078)	(0.076)	(0.067)
Past welfare				0.010
				(0.043)
HH income				0.000
				(0.001)
Religiosity				−0.021
				(0.035)
State safety net generosity				0.020**
				(0.009)
Conservative				−0.213***
				(0.042)
house appreciation				−0.001
				(0.001)
N	511	511	506	437
Std. Errors	robust	robust	robust	state-clustered
R^2	0.02	0.02	0.09	0.17
AIC	362.4	364.0	340.1	273.7

* p <0.1, ** p <0.05, *** p <0.01
Standard errors in parentheses. Pre-registered covariates include age, gender race, job tenure, hourly status, full time status, college degree, and main job.

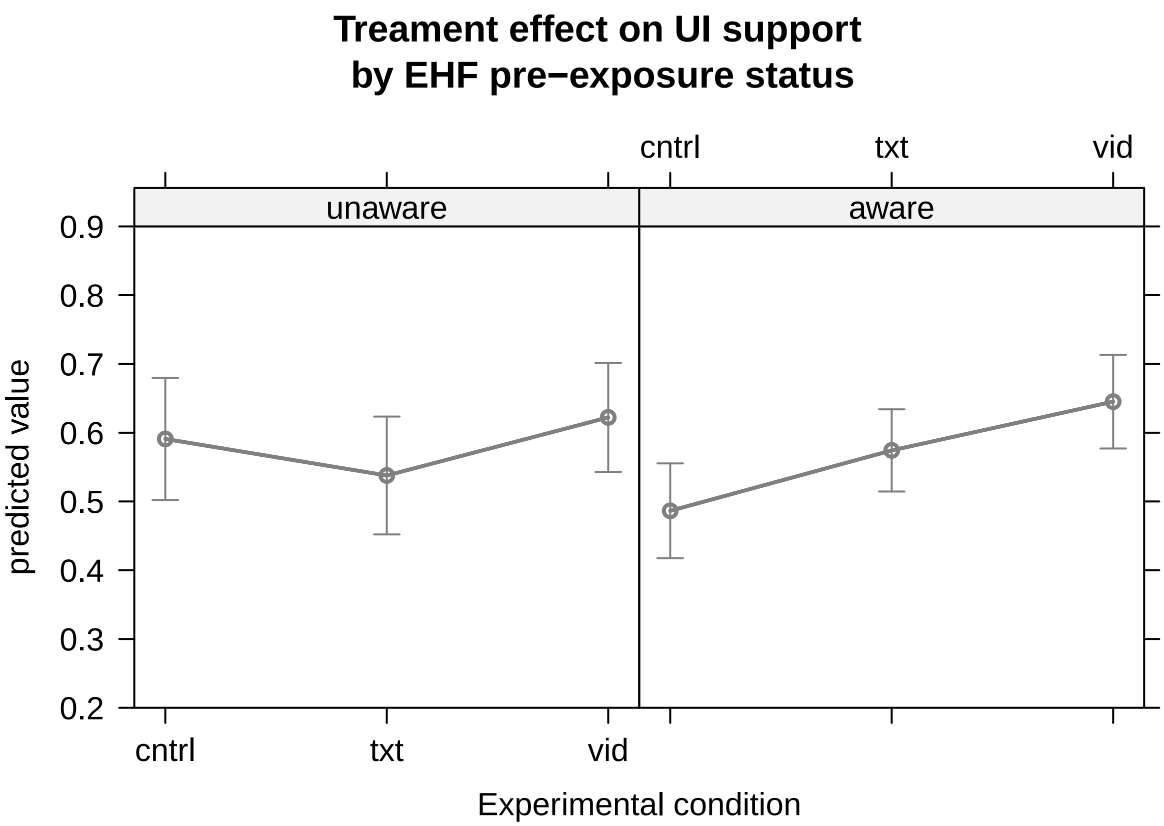


Figure 5: Interpreting treatment effects on support for government aid to unemployed

as well as predictors relevant to the substitution hypothesis.

In Study 2, we did see some evidence that those pre-exposed to the EHF reacted to treatment differently than the unexposed, consistent with H2a, but the effects observed were not in the expected direction under H2. We see no evidence consistent with the hypothesis that employer-provided "private welfare" crowds out support for the social safety net. If anything, the EHF treatment increases support for government action to support the unemployed.

4.3.2 Study 3

The survey experiment for the Study 3 is similar to that from Study 2. We pre-registered a plan to examine whether treatment effects differ based on pre-exposure to the ACNT, measured using the same question format as in Study 2. Nevertheless, we modified the Home Depot experiment for Study 3. First, based on weak performance of the text treatment from the Home Depot, we use only video vignettes for the Walmart sample, omitting the text treatment. Unfortunately, Walmart does not produce video or other testimonials about the ACNT. To construct a comparable treatment for the Walmart sample, we produced an original video using the Homer Fund treatment as both script and audio-visual template. We adapted the video to the Walmart context and used a paid actor; Figure 3, panel (b) displays a still image from the beginning of the video.[28]

Second, to address concerns that findings from Study 2 might be a "video effect", as opposed the effect of messages about the EHF, Study 3 includes a placebo arm. Respondents in the placebo condition watched a video in which the same hired actor reads a Walmart press release about the company's acquisition of Vizio.[29] In Study 3, respondents were randomly assigned with equal probability to either treatment or control. Within the control condition, respondents were randomly divided between the pure control and the placebo. The pure controls saw no additional information, just as in Study 2.[30] Manipulation checks (see Appendix G) confirm that the video treatment did indeed increase respondent awareness of the ACNT, as expected, whereas the placebo had no effect. Thus we display results for binary treatment/control in the analysis below.

Third, we modified the outcomes we study in two ways. In the Walmart survey we again used the GSS question battery and asked respondents about the level of government responsibility for

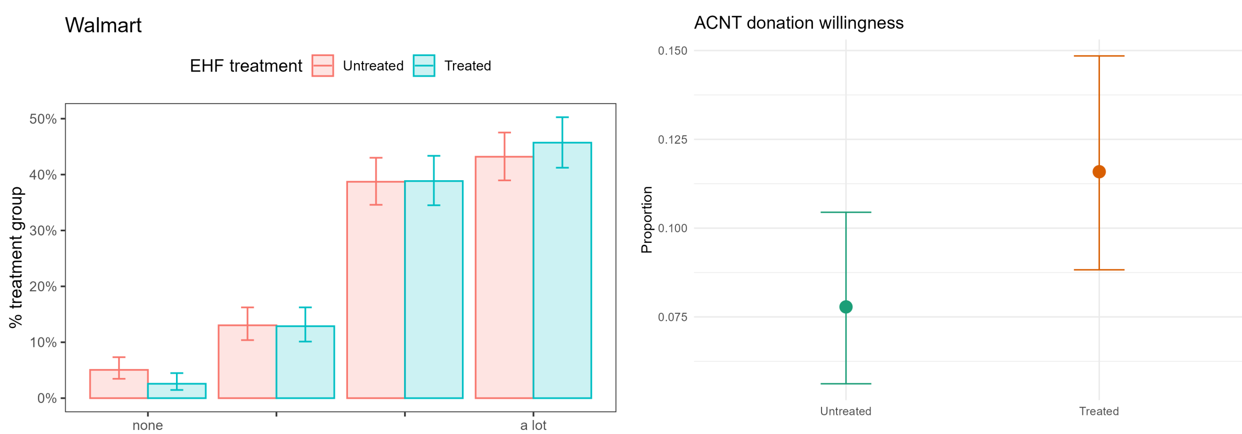
[28]The paid actor exhibited the same gender (male) and racial (white) expression as in the Homer Fund video. The actor was also viewed as approximately the same age as the spokesperson in the Homer Fund video, according to mTurk coders. The ACNT-adapted video was 41 seconds shorter than the Homer Fund video. We achieved this by shortening or removing some interstitial and panning shots, not through any reduction in the spoken script beyond those parts that were only relevant to the Home Depot.

[29]Videos can be seen here: placebo video; treatment (solidarity card); treatment (charity card).

[30]As part of Study 3, we pre-registered a sub-experiment in which we randomly vary the initial framing of the treatment video. Specifically, the "card" visible on the screen before the respondent clicks "play" differs, with the *solidarity* framing displaying text emphasizing supporting other Walmart workers and the and the *charity* framing displaying text emphasizing charitable donations to help those in need. The content of the video vignette was otherwise identical across treatment groups; see previous footnote for links to both versions of the treatment video. There were no detectable differences between the two different framings of the treatment video, so we do not report on this in the main text. Regression results demonstrating this for key outcomes appears in Appendix S of the companion piece (Ahlquist 2025).

ensuring the living standards of the unemployed. But to more closely link outcome measurement with EHFs in Study 3, we substituted a question asking about government responsibility for "people confronting short-term hardships" for the childcare question in Study 2. We also created a behavioral indicator of donation willingness to the Walmart EHF. At the end of Study 3, respondents had the opportunity to click on a link to take them to a page where they could donate to the ACNT. Based on past research, we expect those exposed to the emotional appeal in the video to be more willing to donate (Andreoni and Rao 2011; Bekkers and Wiepking 2011a). We examine whether the generosity of the state safety net moderates this treatment effect.

4.3.2.1 Attitudes toward social insurance and emergency government aid The right panel of Figure 4 displays the Study 3 response distribution for the unemployment support outcome while the left panel of Figure 6 displays the response distribution for the emergency aid outcome. Over 80% of the Walmart sample agreed that the government should take at least some responsibility for the living standards of the unemployed, a much higher rate than among the Home Depot workers. The Walmart sample exhibited a similar level of support for government aid to those facing hardship. Among the Walmart sample, there is no treatment effect apparent for either outcome. Although the pattern of responses for UI for Walmart is similar to Study 2: those in the treatment group were less likely to say that the government has no responsibility for the unemployed, contrary to the substitution hypothesis.



(a) emergency aid (b) ACNT donation

Figure 6: Additional Walmart outcomes, by EHF treatment status. Vertical bars are 95% confidence intervals.

Table 5 displays regression results that reinforce the null relationship in Figures 4 and 6. There is no evidence of a treatment effect for either outcome. Pre-exposure to the ACNT and additional covariates make no difference in this conclusion. In the "expanded" model we continue to see that conservatives are less supportive of government activity to support the unemployed and those facing temporary hardship. Walmart workers who have seen their home values appreciate are less supportive of UI but there is no relationship between home appreciation and support for government aid to those facing temporary hardship. More religious respondents are significantly less supportive of aid to those in temporary need. The generosity of the State-level safety net has no connection to respondents attitudes for either outcome.

Table 5: Support for unemployment insurance and emergency government aid (Walmart)

	DV: Unemployed				DV: Emergency			
	Base	Pre-exposed	Pre-registered	Expanded	Base	Pre-exposed	Pre-registered	Expanded
Treated	0.016	0.018	0.016	0.007	0.019	0.012	0.012	−0.002
	(0.014)	(0.017)	(0.018)	(0.016)	(0.013)	(0.017)	(0.017)	(0.015)
Pre-exposed		0.003	0.007	−0.001		−0.007	−0.005	−0.015
		(0.020)	(0.020)	(0.014)		(0.019)	(0.020)	(0.020)
Treated x pre-exposed		−0.005	−0.005	0.006		0.017	0.014	0.026
		(0.028)	(0.028)	(0.028)		(0.027)	(0.027)	(0.025)
Past welfare				0.013				0.016
				(0.016)				(0.017)
HH income				0.000				0.000
				(0.000)				(0.000)
Religiosity				0.003				−0.017
				(0.013)				(0.011)
State safety net generosity				−0.008				−0.004
				(0.006)				(0.007)
Conservative				−0.061***				−0.057***
				(0.017)				(0.020)
House appreciation				0.000***				0.000
				(0.000)				(0.000)
Std Errors	robust	robust	robust	state-clustered	robust	robust	robust	state-clustered
N	980	980	980	913	980	980	980	913
R^2	0.00	0.00	0.01	0.05	0.00	0.00	0.01	0.04
AIC	−222.9	−219.0	−216.2	−209.3	−320.9	−317.3	−313.4	−299.2

* $p < 0.1$, ** $p < 0.05$, *** $p < 0.01$ Standard errors in parentheses. Pre-registered covariates include age, gender, race, job tenure, hourly status, full-time status, college degree, and main job. Both DV panels report identical specifications.

Walmart has a substantially less generous EHF than the Home Depot. Given the findings in Studies 1 and 2, the null findings in Study 3 are unsurprising. Exposing Walmart workers to videos about their employers' EHF had no effect on their attitudes toward unemployment benefits or government emergency aid, although Walmart workers were, overall, much more supportive of these programs.

4.3.2.2 Willingness to donate to the Walmart EHF

Study 3 included an opportunity to click a link to donate to the ACNT. As the right panel in Figure 6 shows, there is a low baseline willingness to take this action of 8% in the Walmart sample. Nevertheless, those seeing the EHF video were about 4 percentage points (or 35%, $p < 0.05$) more likely to take an action to donate.

Table 6 presents regression-based estimates of treatment effects, using linear probability models.[31] Treatment significantly increases willingness to take an action to donate. There is no evidence that either state-level generosity or ACNT pre-exposure moderates these treatment effects.[32] The inclusion of additional covariates does nothing to alter these conclusions.

The ACNT treatment did significantly increase willingness to donate, as expected, indicating that the treatment did work. But there is no evidence to support the claim that welfare generosity has

[31] See H for estimates using logistic regression and Firth regression to address concerns with rare events. Estimated treatment effects are quite similar across models.

[32] Hierarchical logit models including state-level random effects and random slopes produce the same conclusion.

Table 6: ACNT donation willingness (Walmart sample)

	Base	State Generosity	Pre-exposure	Pre-registered	Expanded
Treated	0.038**	0.075**	0.031	0.033	0.027
	(0.019)	(0.036)	(0.025)	(0.025)	(0.030)
Pre-exposed			−0.021	−0.017	−0.024
			(0.024)	(0.024)	(0.021)
Treated x pre-exposed			0.017	0.011	0.019
			(0.039)	(0.038)	(0.045)
State safety net generosity		0.011			−0.006
		(0.013)			(0.011)
Treated x generosity		−0.029			
		(0.023)			
Past welfare					−0.005
					(0.022)
HH income					0.000
					(0.000)
Religiosity					0.037*
					(0.022)
Conservative					−0.039*
					(0.021)
house appreciation					0.000
					(0.000)
Std Errors	robust	state-clustered	robust	robust	state-clustered
N	980	980	980	980	913
AIC	386.8	388.9	390.2	389.6	409.9

* $p < 0.1$, ** $p < 0.05$, *** $p < 0.01$ Standard errors in parentheses. Pre-registerd covariates include age, gender race, job tenure, hourly status, full time status, college degree, and main job.

any effect on the willingness of workers to donate to an EHF program. Across all three studies, we see no evidence consistent with either H1 or H2.

5 Discussion and Conclusion

There is a significant scholarly tradition arguing about whether private resources and charitable activities are substitutes for public welfare and social insurance. These literatures have virtually ignored the workplace, even though employers have tremendous flexibility to introduce or modify benefit plans and workplace programs that address a variety of economic risks. EHFs are one such tax-incentivized program that has received virtually no systematic study. This oversight is all the more remarkable as EHFs clearly echo both an earlier era of welfare capitalism and past mutual aid initiatives that produced and modern labor unions. Do these programs influence workers' attitudes and behaviors in ways consistent with the "substitution hypothesis"?

Based on three pre-registered studies, our answer is a definitive "no." In Study 1, we found little evidence that state-level welfare effort predicts support for or donation willingness to EHFs. In Studies 2 and 3, using survey experiments, there was no evidence that EHFs reduce support for government unemployment benefits or emergency aid, regardless of the substantial differences in generosity across the Home Depot and Walmart programs. In the Home Depot sample, the treatment induced a modest but significant *increase* in support for government-provided UI among workers already aware of the EHF. This may be because these workers having a better understanding of the limitations of the EHF program. In Study 3, the treatment effect on EHF donations did not vary by state welfare generosity, contrary to expectations. Findings in these surveys are inconsistent with a simple “private options crowd out public benefits” story.

This study is not designed to speak to mechanisms, but findings do point toward future research. The lack of any substitution effect between EHFs and support for public assistance suggests that EHFs may represent a reconfiguration of the moral economy of low-wage work. Strong worker interest in EHFs may reflect pessimism about the fraying safety net in the US. Rather than a substitute for social insurance and assistance, EHFs may be low-cost signals of corporate values and selective benevolence. Direct comparison between attitudes toward government- and employer-provided benefits would be instructive here.

An alternative interpretation is that EHFs are simply too new, too uncertain, or too small to matter yet. More probing, perhaps through interviews or analysis of online worker forums could shed light here. EHFs also emerged in the US, where the social safety net is particularly weak, inequality is stark, and regional variation in economic prospects continue to diverge. Cross national research in worker interest in EHF-type programs might speak to interesting variation in this regard.

Finally, political scientists have paid limited attention to how knowledge of government programs affects private charitable behavior, despite theoretical reasons to expect that information constraints may moderate crowding out effects. This represents an important avenue for future research, particularly relevant to understanding how employee knowledge of government programs might affect workplace charity.

From a policy perspective, EHFs appear to resonate with a felt need among low-wage retail workers in the US today. EHFs programs operate in the shadow of eroded public welfare institutions, yet they are funded in part by favorable tax treatment. EHFs may present a private, partial complement to a weakening or slow public system. By locating mutual aid programs in the firm, program participation might be higher and the EHF can benefit from corporate resources. Applying for an EHF grant may be easier or less stigmatizing than applying for government assistance and the turn around time for benefits could be much faster in times of acute need.[33] Are the benefits these programs provide worth the tax expenditures? How do these programs change when workers themselves have a voice in how they are run?

[33]The Home Depot claims that grants are processed within 7 business days of application receipt, far faster than welfare or unemployment benefits, which also require waiting periods in many states.

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A Deviations from pre-analysis plans

The research in this paper was pre-registered with OSF as osf.io/fhq93

Table 7: Summary of Deviations from Pre-Analysis Plan

Study	Change	Rationale
All	Covariate specifications	In some of the covariate specifications, we include measures of partisanship, political ideology, home ownership, and state of residence. These are included in addition to pre-registered specifications to specifically connect with the existing literature.
Study 1	Survey subjects	In the PAP, we planned a survey of Costco workers, as Costco is the only top-10 retailer without an EHF. Recruiting a meaningful sample of Costco workers through social media proved impossible. We turned to a sample of general retail workers instead. Measurement and analysis of the survey follows the pre-registered plan.
Study 2	HTE by pre-exposure	Analyzing HTE by pre-exposure (alternatively, analyzing compliers and always-takers) was not part of the initial PAP. Its importance to understanding the experiment is obvious so the analysis is included alongside the pre-registered analysis.
Study 3	Framing results not reported in main text	We pre-registered a framing experiment within the treatment arm for Study 3. For those in the treatment arm, we randomized between two different text statements on the screen before the respondent pushes 'play'. These messages are the 'charity' and 'solidarity' framing, respectively. There were no detectable differences between charity and solidarity treatment groups for any outcome, so we omit discussion in the main text and pool all treated respondents. These results are presented in Appendix S of Ahlquist (2025).

Study 3	Follow-up survey	Respondents were sent emails inviting them into the follow-up survey one week after completion, offering a second raffle entry. They were contacted a maximum of three times. We received valid follow-up responses from 157 (16%) of those completing the main survey. Analysis appears below, but low response rates and small samples limit the power and reliability of results.

B General retail survey survey details

Table 8: **Sample summary statistics**

Characteristic	**N = 1,004**
EHF awareness (list)	230 (23%)
age_clean	38 (30, 51)
male	510 (51%)
main job	946 (94%)
job tenure	
3 or more years	457 (45%)
At least 1 year but less than 2 years	193 (19%)
At least 2 years but less than 3 years	139 (14%)
At least 6 months but less than 1 year	124 (12%)
Less than 6 months	92 (9.1%)
PoC/nonwhite	376 (37%)
full time	645 (64%)
hourly	837 (83%)
college degree	287 (29%)
union member	146 (16%)
(missing/not gathered)	69
EHF awareness (direct)	
No	625 (62%)
Unsure	65 (6.4%)
Yes	315 (31%)
deal with emergency expense	
I am certain I could come up with the full $400	391 (39%)
I am certain I could not come up with $400	116 (12%)
I could probably come up with $400	307 (31%)
I could probably not come up with $400	191 (19%)
accept new job	
Extremely likely	136 (13%)
Not at all likely	234 (23%)
Not very likely	227 (23%)
Somewhat likely	268 (27%)
Very likely	140 (14%)
attachment index	-0.25 (-1.19, 1.03)

[1] n (%); Median (Q1, Q3)

Table 9: *summary table (cont'd)*

Characteristic	**N = 1,004**
loyalty to coworkers	
A lot of loyalty	332 (33%)
No loyalty at all	90 (8.9%)
Only a little loyalty	160 (16%)
Some loyalty	423 (42%)
loyalty to employer	
A lot of loyalty	331 (33%)
No loyalty at all	118 (12%)
Only a little loyalty	211 (21%)
Some loyalty	344 (34%)
recommend employer	
Certainly would recommend	349 (35%)
Definitely would not recommend	71 (7.1%)
Might not recommend	102 (10%)
Might recommend	395 (39%)
Not sure	87 (8.7%)
union vote	
Against the union	205 (20%)
For the union	439 (44%)
Leaning against voting for the union	29 (2.9%)
Leaning toward voting for the union	64 (6.4%)
Not sure	267 (27%)
hardship support	
A little responsibility	200 (20%)
A lot of responsibility	327 (33%)
No responsibility	52 (5.2%)
Some responsibility	425 (42%)
pension support	
A little responsibility	103 (10%)
A lot of responsibility	579 (58%)
No responsibility	26 (2.6%)
Some responsibility	296 (29%)
UI support	
A little responsibility	224 (22%)
A lot of responsibility	329 (33%)
No responsibility	56 (5.6%)
Some responsibility	394 (39%)

[1] n (%)

C Home Depot suggested Homer Fund contribution rates

HOMER FUND

CONSULT DONATE RESOURCES CONTACT

(*one-time payroll donations have a $10 minimum*)

CLICK HERE to support your fellow associates via credit card donation

Review our suggested levels of giving below:

JOB TITLE	LEVEL	PER CHECK	ANNUAL AMOUNT
EVPs	Leaders' Circle	$400+	$10,000+
SVPs/Division Presidents	Partners' Circle	$200 - $399	$5,000 to $9,999
Vice Presidents	Giving Back Circle	$100 - $199	$2,500 to $4,999
Directors/DMs	Taking Care Circle	$40 - $99	$1,000 to $2,499
Managers	Associates' Circle	$20 - $39	$500 - $ 999
Associates	Contributors' Circle	$5 - $19	$125 - $499

Figure 7: Suggested Homer Fund donation levels as of 2025

D Home Depot Survey summary table (unweighted)

The unweighted sample is older and skewed slightly male relative to reported Facebook demographics for Home Depot workers.[34] The sample is 79% white.

[34]Our sample is 56% male and has a median age of 50 compared to Facebook reported values of 53% male with a median age in the 30-39 interval. The weighted sample is 52% male with a median age of 38.

Table 10: **Sample summary statistics by treatment group**

Characteristic	**cntrl** N = 164	**txt** N = 197	**vid** N = 154
EHF awareness (list)	98 (60%)	120 (61%)	94 (61%)
EHF awareness (cntrl)			
Don't know	17 (10%)	0 (NA%)	0 (NA%)
No	9 (5.5%)	0 (NA%)	0 (NA%)
Yes	138 (84%)	0 (NA%)	0 (NA%)
(missing/not gathered)	0	197	154
age	52 (31, 62)	47 (32, 61)	51 (34, 62)
male	98 (60%)	109 (56%)	81 (53%)
(missing/not gathered)	0	1	1
main job	147 (90%)	176 (89%)	139 (90%)
(missing/not gathered)	1	0	0
job tenure			
Less than 6 months	10 (6.1%)	13 (6.6%)	7 (4.5%)
At least 6 months but less than 1 year	16 (9.8%)	8 (4.1%)	12 (7.8%)
At least 1 year but less than 2 years	25 (15%)	25 (13%)	30 (19%)
At least 2 years but less than 3 years	15 (9.1%)	15 (7.6%)	16 (10%)
3 or more years	98 (60%)	136 (69%)	89 (58%)
PoC/nonwhite	38 (23%)	33 (17%)	37 (24%)
(missing/not gathered)	1	3	1
full time	98 (60%)	138 (70%)	91 (59%)
hourly	158 (96%)	186 (94%)	153 (99%)
college degree	37 (23%)	43 (22%)	29 (19%)
knows EHF recipient	0 (NA%)	120 (61%)	82 (53%)
(missing/not gathered)	164	1	0
applied to EHF	0 (NA%)	45 (23%)	35 (23%)
(missing/not gathered)	164	0	0
received EHF grant	22 (13%)	27 (14%)	22 (14%)
donated to EHF			
Don't know	7 (4.3%)	6 (3.0%)	11 (7.1%)
No	36 (22%)	25 (13%)	33 (21%)
Yes	120 (74%)	166 (84%)	110 (71%)
(missing/not gathered)	1	0	0

[1] n (%); Median (Q1, Q3)

Table 11: *summary table (cont'd)*

Characteristic	**cntrl** N = 164	**txt** N = 197	**vid** N = 154
loyalty to coworkers			
No loyalty at all	14 (8.6%)	10 (5.1%)	6 (3.9%)
Only a little loyalty	16 (9.8%)	27 (14%)	11 (7.1%)
Some loyalty	65 (40%)	83 (42%)	52 (34%)
A lot of loyalty	68 (42%)	77 (39%)	85 (55%)
(missing/not gathered)	1	0	0
loyalty to employer			
No loyalty at all	23 (14%)	22 (11%)	8 (5.2%)
Only a little loyalty	25 (16%)	27 (14%)	24 (16%)
Some loyalty	45 (28%)	81 (42%)	44 (29%)
A lot of loyalty	67 (42%)	65 (33%)	77 (50%)
(missing/not gathered)	4	2	1
recommend employer			
Definitely would not recommend	24 (15%)	20 (10%)	6 (3.9%)
Might not recommend	11 (6.7%)	21 (11%)	10 (6.5%)
Not sure	10 (6.1%)	16 (8.2%)	9 (5.8%)
Might recommend	34 (21%)	46 (23%)	43 (28%)
Certainly would recommend	85 (52%)	93 (47%)	86 (56%)
(missing/not gathered)	0	1	0
union vote			
Against the union	78 (48%)	99 (50%)	66 (43%)
For the union	41 (25%)	43 (22%)	24 (16%)
Not sure	45 (27%)	55 (28%)	63 (41%)
(missing/not gathered)	0	0	1
UI support			
No responsibility	34 (21%)	31 (16%)	14 (9.2%)
A little responsibility	39 (24%)	53 (27%)	37 (24%)
Some responsibility	52 (32%)	57 (29%)	51 (33%)
A lot of responsibility	39 (24%)	53 (27%)	51 (33%)
(missing/not gathered)	0	3	1
pension support			
No responsibility	17 (10%)	18 (9.3%)	9 (5.9%)
A little responsibility	17 (10%)	23 (12%)	13 (8.6%)
Some responsibility	40 (25%)	60 (31%)	44 (29%)
A lot of responsibility	88 (54%)	93 (48%)	86 (57%)
(missing/not gathered)	2	3	2
childcare support			
No responsibility	36 (22%)	37 (19%)	24 (16%)
A little responsibility	29 (18%)	28 (15%)	20 (13%)
Some responsibility	48 (29%)	76 (39%)	64 (42%)
A lot of responsibility	50 (31%)	52 (27%)	43 (28%)
(missing/not gathered)	1	4	3

[1] n (%)

E Walmart survey summary table (unweighted)

Table 12: **Sample summary statistics by treatment group**

Characteristic	**ctrl** N = 260	**vid0** N = 254	**vidChar** N = 237	**vidSolid** N = 229
EHF awareness (pre-treatment)	116 (45%)	97 (38%)	91 (38%)	85 (37%)
age				
18-45	115 (44%)	114 (45%)	132 (56%)	103 (45%)
45-65+	145 (56%)	140 (55%)	105 (44%)	126 (55%)
male	55 (21%)	55 (22%)	63 (27%)	47 (21%)
main job	224 (86%)	232 (91%)	207 (87%)	205 (90%)
job tenure				
3 or more years	124 (48%)	129 (51%)	116 (49%)	106 (46%)
At least 1 year but less than 2 years	49 (19%)	43 (17%)	41 (17%)	46 (20%)
At least 2 years but less than 3 years	25 (9.6%)	27 (11%)	24 (10%)	24 (10%)
At least 6 months but less than 1 year	32 (12%)	26 (10%)	27 (11%)	23 (10%)
Less than 6 months	30 (12%)	29 (11%)	29 (12%)	30 (13%)
PoC/nonwhite	51 (20%)	49 (19%)	52 (22%)	39 (17%)
full time	196 (75%)	193 (76%)	187 (79%)	189 (83%)
hourly	248 (95%)	247 (97%)	227 (96%)	223 (97%)
college degree	24 (9.2%)	25 (9.8%)	18 (7.6%)	23 (10%)
EHF awareness (post-treatment)				
No	98 (56%)	109 (61%)	98 (54%)	107 (56%)
Unsure	14 (8.0%)	15 (8.4%)	8 (4.4%)	17 (8.9%)
Yes	63 (36%)	54 (30%)	75 (41%)	67 (35%)
(missing/not gathered)	85	76	56	38
applied to EHF				
Don't know	4 (2.3%)	4 (2.2%)	3 (1.7%)	0 (0%)
No	143 (82%)	155 (87%)	149 (82%)	168 (88%)
Yes	28 (16%)	19 (11%)	29 (16%)	23 (12%)
(missing/not gathered)	85	76	56	38
received EHF grant	19 (59%)	8 (35%)	18 (56%)	14 (61%)
(missing/not gathered)	228	231	205	206
donated to EHF	16 (9.1%)	15 (8.4%)	29 (16%)	18 (9.4%)
(missing/not gathered)	85	76	56	38

[1] n (%)

Table 13: *summary table (cont'd)*

Characteristic	**ctrl** N = 260	**vid0** N = 254	**vidChar** N = 237	**vidSolid** N = 229
loyalty to coworkers				
A lot of loyalty	72 (28%)	85 (33%)	77 (32%)	76 (33%)
No loyalty at all	29 (11%)	15 (5.9%)	22 (9.3%)	19 (8.3%)
Only a little loyalty	43 (17%)	48 (19%)	36 (15%)	42 (18%)
Some loyalty	116 (45%)	106 (42%)	102 (43%)	92 (40%)
loyalty to employer				
A lot of loyalty	62 (24%)	72 (28%)	57 (24%)	65 (28%)
No loyalty at all	65 (25%)	55 (22%)	46 (19%)	49 (21%)
Only a little loyalty	50 (19%)	51 (20%)	51 (22%)	49 (21%)
Some loyalty	83 (32%)	76 (30%)	83 (35%)	66 (29%)
recommend employer				
Certainly would recommend	90 (35%)	97 (38%)	80 (34%)	80 (35%)
Definitely would not recommend	44 (17%)	31 (12%)	33 (14%)	34 (15%)
Might not recommend	31 (12%)	29 (11%)	34 (14%)	30 (13%)
Might recommend	72 (28%)	72 (28%)	75 (32%)	57 (25%)
Not sure	23 (8.8%)	25 (9.8%)	15 (6.3%)	28 (12%)
union vote				
Against the union	50 (19%)	40 (16%)	29 (12%)	29 (13%)
For the union	113 (43%)	112 (44%)	117 (49%)	103 (45%)
Leaning against voting for the union	7 (2.7%)	4 (1.6%)	4 (1.7%)	3 (1.3%)
Leaning toward voting for the union	7 (2.7%)	11 (4.3%)	5 (2.1%)	6 (2.6%)
Not sure	83 (32%)	87 (34%)	82 (35%)	88 (38%)
hardship support				
A little responsibility	30 (12%)	37 (15%)	36 (15%)	24 (10%)
A lot of responsibility	105 (40%)	117 (46%)	104 (44%)	109 (48%)
No responsibility	15 (5.8%)	11 (4.3%)	7 (3.0%)	5 (2.2%)
Some responsibility	110 (42%)	89 (35%)	90 (38%)	91 (40%)
pension support				
A little responsibility	10 (3.8%)	8 (3.1%)	13 (5.5%)	11 (4.8%)
A lot of responsibility	192 (74%)	192 (76%)	175 (74%)	180 (79%)
No responsibility	5 (1.9%)	2 (0.8%)	2 (0.8%)	5 (2.2%)
Some responsibility	53 (20%)	52 (20%)	47 (20%)	33 (14%)
UI support				
A little responsibility	35 (13%)	49 (19%)	46 (19%)	30 (13%)
A lot of responsibility	92 (35%)	97 (38%)	82 (35%)	95 (41%)
No responsibility	21 (8.1%)	13 (5.1%)	8 (3.4%)	11 (4.8%)
Some responsibility	112 (43%)	95 (37%)	101 (43%)	93 (41%)

[1] n (%)

Table 14: **Sample summary statistics by treatment group**

Characteristic	**ctrl** N = 239	**vid0** N = 234	**vidChar** N = 233	**vidSolid** N = 212
EHF awareness (pre-treatment)	106 (44%)	89 (38%)	83 (36%)	75 (35%)
age				
18-45	111 (46%)	109 (47%)	132 (57%)	94 (44%)
45-65+	128 (54%)	125 (53%)	101 (43%)	118 (56%)
male	88 (37%)	87 (37%)	103 (44%)	75 (35%)
main job	203 (85%)	213 (91%)	201 (86%)	188 (89%)
job tenure				
3 or more years	116 (49%)	125 (53%)	112 (48%)	98 (46%)
At least 1 year but less than 2 years	43 (18%)	39 (17%)	39 (17%)	44 (21%)
At least 2 years but less than 3 years	24 (10%)	21 (9.0%)	24 (10%)	24 (11%)
At least 6 months but less than 1 year	31 (13%)	25 (11%)	28 (12%)	21 (9.8%)
Less than 6 months	25 (10%)	24 (10%)	30 (13%)	25 (12%)
PoC/nonwhite	53 (22%)	47 (20%)	55 (24%)	39 (18%)
full time	179 (75%)	179 (76%)	181 (78%)	174 (82%)
hourly	225 (94%)	226 (97%)	222 (95%)	204 (96%)
college degree	29 (12%)	26 (11%)	24 (10%)	24 (11%)
EHF awareness (post-treatment)				
No	84 (54%)	101 (61%)	97 (54%)	101 (57%)
Unsure	14 (9.2%)	14 (8.5%)	8 (4.3%)	16 (8.8%)
Yes	58 (37%)	50 (30%)	75 (42%)	62 (35%)
(missing/not gathered)	83	69	54	33
applied to EHF				
Don't know	3 (2.1%)	5 (3.1%)	3 (1.5%)	0 (0%)
No	129 (83%)	142 (86%)	154 (86%)	160 (89%)
Yes	23 (15%)	18 (11%)	22 (12%)	19 (11%)
(missing/not gathered)	83	69	54	33
received EHF grant	16 (60%)	8 (36%)	14 (57%)	11 (58%)
(missing/not gathered)	213	211	208	192
donated to EHF	15 (9.5%)	13 (8.0%)	30 (17%)	17 (9.5%)
(missing/not gathered)	83	69	54	33

[1] n (%)

Table 15: *summary table (cont'd)*

Characteristic	**ctrl** N = 239	**vid0** N = 234	**vidChar** N = 233	**vidSolid** N = 212
loyalty to coworkers				
A lot of loyalty	63 (26%)	75 (32%)	72 (31%)	65 (31%)
No loyalty at all	30 (13%)	15 (6.6%)	24 (10%)	17 (8.1%)
Only a little loyalty	40 (17%)	44 (19%)	40 (17%)	40 (19%)
Some loyalty	106 (44%)	100 (43%)	96 (41%)	90 (43%)
loyalty to employer				
A lot of loyalty	54 (22%)	62 (26%)	51 (22%)	57 (27%)
No loyalty at all	65 (27%)	55 (23%)	51 (22%)	46 (22%)
Only a little loyalty	45 (19%)	45 (19%)	51 (22%)	49 (23%)
Some loyalty	76 (32%)	72 (31%)	80 (34%)	60 (29%)
recommend employer				
Certainly would recommend	76 (32%)	85 (36%)	69 (30%)	73 (34%)
Definitely would not recommend	40 (17%)	30 (13%)	34 (15%)	31 (14%)
Might not recommend	31 (13%)	29 (12%)	39 (17%)	28 (13%)
Might recommend	69 (29%)	65 (28%)	76 (32%)	54 (25%)
Not sure	22 (9.3%)	26 (11%)	15 (6.7%)	27 (13%)
union vote				
Against the union	49 (21%)	39 (17%)	32 (14%)	27 (13%)
For the union	107 (45%)	103 (44%)	118 (51%)	94 (44%)
Leaning against voting for the union	5 (2.2%)	5 (2.0%)	3 (1.4%)	2 (1.0%)
Leaning toward voting for the union	6 (2.3%)	11 (4.6%)	4 (1.6%)	5 (2.5%)
Not sure	72 (30%)	77 (33%)	76 (33%)	84 (40%)
hardship support				
A little responsibility	30 (12%)	38 (16%)	38 (16%)	22 (11%)
A lot of responsibility	98 (41%)	104 (44%)	95 (41%)	95 (45%)
No responsibility	13 (5.5%)	11 (4.6%)	8 (3.4%)	4 (1.9%)
Some responsibility	98 (41%)	82 (35%)	92 (39%)	91 (43%)
pension support				
A little responsibility	7 (2.9%)	8 (3.6%)	17 (7.3%)	10 (4.5%)
A lot of responsibility	174 (73%)	176 (75%)	166 (71%)	165 (78%)
No responsibility	4 (1.6%)	1 (0.5%)	3 (1.3%)	4 (1.8%)
Some responsibility	54 (23%)	49 (21%)	47 (20%)	34 (16%)
UI support				
A little responsibility	33 (14%)	44 (19%)	48 (21%)	28 (13%)
A lot of responsibility	90 (38%)	90 (38%)	81 (35%)	86 (41%)
No responsibility	17 (7.3%)	11 (4.7%)	6 (2.8%)	9 (4.2%)
Some responsibility	98 (41%)	89 (38%)	97 (42%)	89 (42%)

[1] n (%)

F Video treatment and placebo similarities

Here we use text, acoustic, and dynamic video embeddings to describe the multimodal similarties between (i) the placebo and treatment videos for Walmart and (ii) the Home Depot and Walmart treatment videos. We assessed treatment video comparability using CLIP ViT-B/32 (**?**), implemented via OpenCLIP on 200 frames per file. For acoustic similarity, we segmented audio tracks into overlapping 10-second windows and encoded them with CLAP (**?**). For transcript comparability we used Sentence-BERT embeddings (**?**) for semantic similarity of the narrated content. In each case, we report the cosine similarity of the mean embeddings as the video-level similarity score.

Exactly as designed, the placebo and treatment videos display high video similarity and acoustic similarlity but low semantic content similarity. The Home Depot and Walmart treatment videos show very high cosine similarity in video, audio, and semantic content.

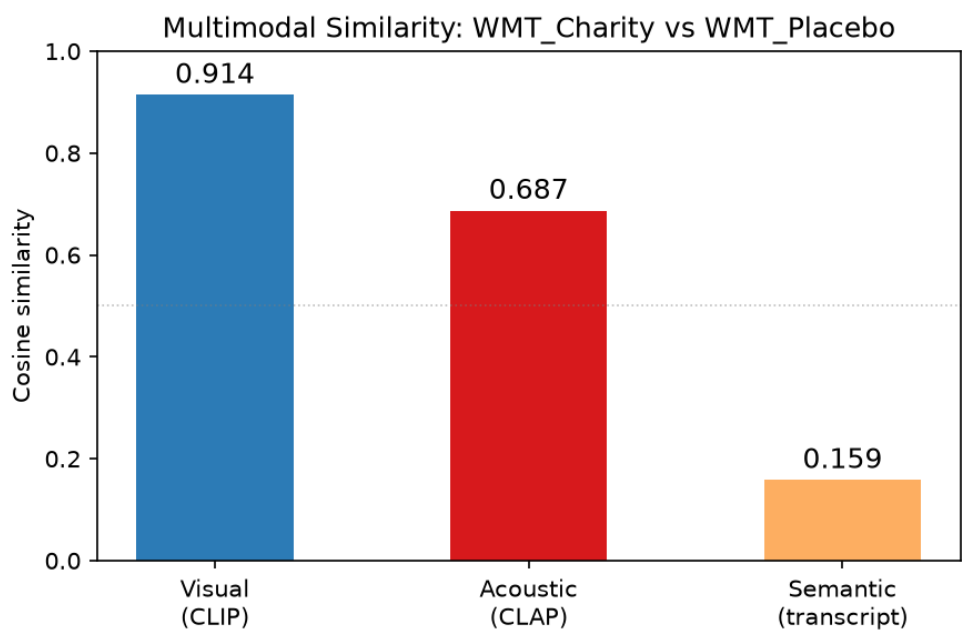

Figure 8: Multimodal Similarity: Walmart Treatment vs. Walmart Placebo

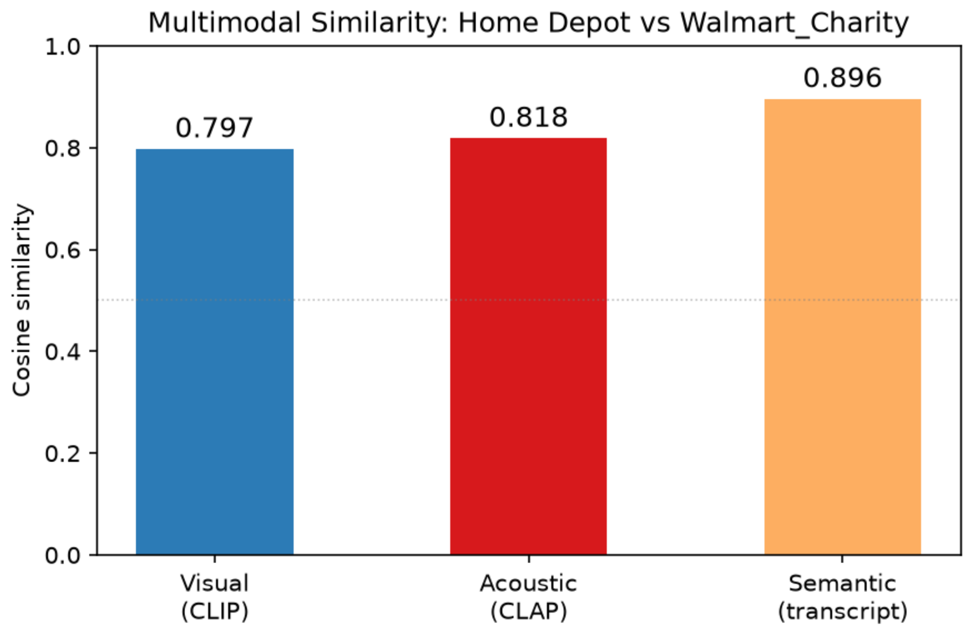

Figure 9: Multimodal Similarity: Walmart Treatment vs. Walmart Placebo

G Manipulation check Study 3

In the Walmart sample, treatment significantly increased the probability that a respondent would correctly state that Walmart has an EHF.

Table 16: OLS regression of treatment on correctly reporting a Walmart EHF (manipulation check)

	(1)	(2)	(3)	(4)
treated	0.118***		0.103***	
	(0.029)		(0.028)	
placebo		−0.061		−0.027
		(0.037)		(0.032)
charity treatment		0.097**		0.067*
		(0.041)		(0.040)
solidarity treatment		0.079*		0.111***
		(0.042)		(0.042)
pre-exposed			0.349***	0.360***
			(0.038)	(0.053)
treat x pre-exposed			0.073	
			(0.058)	
placebo x pre-exposed				−0.028
				(0.076)
charity x pre-exposed				0.135*
				(0.079)
solidarity x pre-exposed				−0.015
				(0.083)
N	980	980	980	980

* $p < 0.1$, ** $p < 0.05$, *** $p < 0.01$
Robust standard errors in parentheses.

H Alternative models for ACNT donation

Table 17: Regression of ACNT donation on treatment

	LPM	logit	Firth
Treated	0.038**	0.440**	0.437**
	(0.019)	(0.219)	(0.218)
N	980	980	980
AIC	386.8	619.3	

* $p < 0.1$, ** $p < 0.05$, *** $p < 0.01$ Robust standard errors for OLS model

I Placebos for UI support (Home Depot)

Table 18: Support for other social policies, OLS regression

	Pension	Childcare
Text treatment	−0.031	−0.028
	(0.060)	(0.066)
Video treatment	0.052	−0.031
	(0.057)	(0.068)
Pre-exposed	0.007	−0.028
	(0.055)	(0.061)
Text x pre-exposed	0.022	0.076
	(0.074)	(0.082)
Video x pre-exposed	−0.013	0.133
	(0.073)	(0.085)
N	508	507
R^2	0.01	0.01
AIC	290.5	400.2

* p <0.1, ** p <0.05, *** p <0.01
Robust standard errors in parentheses.